

DISCHARGE SUMMARY

Hospital Information

Hospital Name: CityCare Multispecialty Hospital
Hospital Registration No: HOSP/AP/556677 (NABH Accredited)
Address: Plot No. 45, Jubilee Hills, Hyderabad – 500033
Contact: +91-40-44556677

Patient & Insurance Details

Patient Name: Ramesh Kumar
Age / Gender: 45 Years / Male
UHID: HSP-556677
Government ID: Aadhaar (XXXX-XXXX-1234)
Health Policy Number: POL-12345678
Claim ID / Reference No: CLM-2024-001
Insurance Company: Medicare Ltd
TPA Name: HealthAssist TPA

Hospitalization Details

Date & Time of Admission: 12-03-2024 at 10:45 AM
Date & Time of Discharge: 17-03-2024 at 04:30 PM
Total Length of Stay: 5 Days
Ward / Room Category: General Ward
Bed Number: GW-12

Chief Complaints at Admission

- High-grade fever for 3 days
- Severe body aches and headache
- Nausea and multiple episodes of vomiting
- Generalized weakness and reduced oral intake

Clinical History

The patient was apparently well three days prior to admission when he developed sudden onset high-grade fever associated with chills, myalgia, retro-orbital pain, and vomiting. There was no history of bleeding manifestations, breathlessness, chest pain, or altered sensorium. No significant past medical history of diabetes, hypertension, cardiac illness, or previous similar episodes was reported.

Examination Findings on Admission

General Condition: Moderately ill, conscious, oriented to time, place, and person.
Pulse Rate: 96/min, regular
Blood Pressure: 110/70 mmHg
Temperature: 102°F
Respiratory Rate: 18/min
Systemic Examination: Cardiovascular, respiratory, and central nervous systems were clinically normal. Mild hepatomegaly was noted on abdominal examination.

Investigations

- Complete Blood Count (CBC)
- Dengue NS1 Antigen Test (Positive)
- Liver Function Tests (SGOT & SGPT elevated)
- Renal Function Tests (Within normal limits)

Diagnosis

Provisional Diagnosis: Viral Fever
Final Diagnosis: Dengue Fever (NS1 Positive) with Mild Thrombocytopenia

Treatment & Hospital Course

The patient was managed conservatively with intravenous fluids, antipyretics, anti-emetics, and close monitoring of platelet counts and vital parameters. Platelet counts were monitored daily and showed gradual improvement. No platelet transfusion was required during the hospital stay. The patient remained hemodynamically stable throughout the hospitalization and showed symptomatic improvement.

Condition at Discharge

The patient was afebrile, clinically stable, and tolerating oral diet well. Vital signs were within normal limits. There were no active complaints at the time of discharge.

Medications at Discharge

- Tablet Paracetamol 650 mg – One tablet orally every 6 hours as needed for fever
- Oral Rehydration Solution (ORS) – As advised

Discharge Advice

- Adequate oral fluid intake
- Avoid strenuous physical activity for one week
- Monitor for warning signs such as bleeding, persistent vomiting, or abdominal pain
- Follow up with CBC after 7 days or earlier if symptoms worsen

Follow-up

Follow-up consultation scheduled after 7 days with treating physician along with repeat blood investigations.

Treating Doctor

Dr. Anil Sharma

MD (General Medicine)

Medical Registration No: AP/MD/334455

Authorized Signatory & Hospital Seal